

Survey Preparation Checklist

Therapy Department · Be ready every day · SNF · 2026

Date Completed: _____

Completed By: _____

Surveyors can arrive any day, unannounced. This checklist should be reviewed weekly — not only when survey is imminent. A department that is survey-ready every day has nothing to fear. HIGH priority items are the most commonly cited therapy deficiencies nationally.

1. Documentation

Most common area of therapy deficiency citations nationally

	!	Task	Surveyors Look For
■	HIGH	All active patients have a current signed physician order for therapy	Surveyors will pull charts and verify orders exist before treatment began
■	HIGH	All evaluations completed within 72 hours of admission or referral	Delayed evals suggest inadequate assessment and failure to meet patient needs
■	HIGH	Every note documents skilled need — not just what was done, but WHY a therapist was required	Notes that read as routine or non-skilled are the top citation trigger
■	HIGH	Goals are functional, measurable, time-bound, and updated as patient progresses	Vague goals ("improve strength") signal lack of individualized care planning
■	MED	Recertification signatures current — no lapsed recerts in active charts	Lapsed recert = gap in coverage. Surveyors check dates carefully
■	MED	Discharge summaries completed within required timeframe for all recent discharges	Late or missing D/C summaries are a documentation deficiency
■	MED	Maintenance program documentation in place for patients on maintenance therapy	Maintenance notes must document skilled need per Jimmo v. Sebelius standard
■	MED	MDS assessments reflect therapy minutes accurately — matches treatment logs	Discrepancy between MDS minutes and treatment logs = major audit finding
■	LOW	Patient/family education documented with content and response noted	Surveyors look for evidence that patients and families are informed participants

2. Individualized Care

F-tag 676 / F-tag 677 — one of the most cited therapy-related deficiencies

	!	Task	Surveyors Look For
■	HIGH	Every patient's treatment plan is individualized — no cookie-cutter programs	Surveyors interview patients and observe treatment — generic programs are cited
■	HIGH	Treatment frequency and duration are clinically justified for each patient	"We always do 5x/week" is not a clinical rationale — document patient-specific need
■	HIGH	Therapy is addressing the patient's specific functional deficits and goals	Treatment must connect directly to the patient's diagnosis and goals — surveyors check
■	MED	Patient preferences and choices are incorporated and documented	Patients have the right to refuse or modify therapy — document discussions
■	MED	Patients can describe their therapy goals when asked by a surveyor	Surveyors will ask patients directly: "Do you know what you're working on in therapy?"

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	!	Task	Surveyors Look For
☐	LOW	Cultural, language, or communication needs addressed in the care plan	<i>Patients with language barriers must have appropriate communication supports documented</i>

3. Falls & Restraints

Therapy is often pulled into fall-related citations — be prepared

	!	Task	Surveyors Look For
☐	HIGH	All patients with fall history have therapy assessment and intervention documented	<i>Surveyors expect therapy to be involved in fall prevention for at-risk patients</i>
☐	HIGH	Post-fall assessments completed by therapy when clinically indicated	<i>Failure to assess after a fall is a direct citation — document every post-fall eval</i>
☐	MED	Fall prevention program in place — therapy contributing to restorative program	<i>Surveyors look for continuity between therapy and restorative fall prevention</i>
☐	MED	Assistive device recommendations documented and device available to patient	<i>Recommending a device the patient doesn't have access to = failed intervention</i>
☐	LOW	Environmental safety recommendations communicated to nursing and documented	<i>Therapy findings that affect patient safety must be communicated interdepartmentally</i>

4. Staffing, Supervision & Competency

	!	Task	Surveyors Look For
☐	HIGH	All treating therapists and assistants have current valid state licensure	<i>Expired license = treatment cannot be billed. Surveyors verify licensure status</i>
☐	HIGH	PT/OT/ST assistants have required supervision documented per state rules	<i>Supervision frequency and documentation requirements vary by state — know yours</i>
☐	HIGH	Students and techs are not billing independently — supervision documented	<i>Student-provided services must be billed under supervising therapist — not independently</i>
☐	MED	New staff competency checklist completed and on file	<i>Surveyors may ask for competency documentation for new or traveling staff</i>
☐	MED	Any contract/travel staff credentialed and oriented to facility policies	<i>Travelers must meet the same standards as staff — orientation must be documented</i>
☐	LOW	Therapy staff aware of their role in the emergency/disaster plan	<i>Staff must know evacuation procedures and their responsibilities in an emergency</i>

5. Care Planning & Interdisciplinary Communication

	!	Task	Surveyors Look For
☐	HIGH	Therapy goals reflected in the interdisciplinary care plan	<i>Therapy plans must be integrated into the overall care plan — not siloed</i>
☐	HIGH	Therapy attending IDT/care conference meetings and input documented	<i>Surveyors look for evidence of interdisciplinary collaboration</i>

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☐	MED	Nursing informed of therapy precautions, weight-bearing status, device use	<i>Critical safety information must be communicated and acknowledged by nursing</i>
☐	MED	Therapy input included in MDS — RN completing MDS has current therapy data	<i>Inaccurate MDS due to poor communication = payment and compliance risk</i>
☐	MED	Family/responsible party updated on therapy progress and goals	<i>Family communication must be documented — surveyors may interview families</i>
☐	LOW	Restorative nursing program receiving proper therapy handoffs	<i>Clear handoff from therapy to restorative demonstrates continuity of care</i>

6. Therapy Gym & Equipment

	!	Task	Surveyors Look For
☐	HIGH	Therapy gym is clean, organized, and free of safety hazards	<i>Surveyors walk through the gym — clutter, spills, or hazards are cited</i>
☐	HIGH	All equipment is clean, in good working order, and safe to use	<i>Broken or poorly maintained equipment is an immediate safety concern</i>
☐	MED	Patient privacy maintained during treatment — screens/curtains available	<i>Privacy during treatment is a patient right — lack of privacy measures is cited</i>
☐	MED	Emergency equipment (oxygen, call system) accessible from therapy gym	<i>Surveyors verify that emergencies can be responded to from all patient care areas</i>
☐	MED	Infection control standards maintained — hand hygiene, equipment disinfection	<i>Infection control lapses in therapy are cited under the facility-wide IC standard</i>
☐	LOW	Therapy supply inventory adequate — no critical supply shortages	<i>Inability to provide needed treatment due to supply issues reflects on care quality</i>

If a Surveyor Approaches You — Know This

1	Be calm and professional. You are not on trial — surveys are a routine compliance process.
2	Answer only what is asked. Do not volunteer additional information or speculate.
3	If you don't know the answer, say "I don't know but I can find out" — never guess.
4	Do not leave a patient unattended to speak with a surveyor. Finish your task first.
5	If asked about a specific patient, you may ask to review the chart before answering.
6	Notify your DOR or administrator immediately if a surveyor interviews you.
7	You have the right to have a supervisor present during an interview — request this if needed.

Total Checked: _____ / 42

HIGH Priority: _____ / 23

Target: 42/42 before survey window